SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Panitumumab + modified FOLFOX-6

First-line RAS wild-type metastatic colorectal cancer · NCCP 00447 · version 7 · 14-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

Schedule
A 14-day cycle: panitumumab, oxaliplatin, folinic acid and fluorouracil start on Day 1, followed by a fluorouracil pump lasting about 46 hours. Arrange disconnection around Day 3. Continue according to benefit and tolerance.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Panitumumab
EGFR-TARGETED ANTIBODY

- ☐ Rash, dry skin, cracks and sore nails
- ☐ Eye inflammation or corneal injury
- ☐ Low magnesium, calcium or potassium
- ☐ Diarrhoea, dehydration and kidney problems
- ☐ Infusion reaction; rare serious lung or skin effects

Oxaliplatin
PLATINUM CHEMOTHERAPY

- ☐ Cold-triggered tingling / throat discomfort
- ☐ Persistent sensory neuropathy, sometimes lasting after treatment
- ☐ Allergic / infusion reaction
- ☐ Low blood counts and infection
- ☐ Nausea, diarrhoea and fatigue
- ☐ Rare serious lung or blood complications

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Discuss RAS selection, skin/nail and eye effects, electrolyte monitoring for eight weeks after completion, infusion reactions and rare serious lung/skin toxicity. Confirm DPD testing, antiemetics, port/PICC care and pump connection/disconnection. Discuss cold-triggered symptoms and potentially lasting neuropathy.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00447 v7.

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